

إسم المريض رباعياً : هند محمد احمد محمد

الحي / المدينة : حدائق القبه

المحافظة : القاهرة

رقم البطاقة : 27406280101221 رقم التليفون : 01011556259

العمر (تاريخ الميلاد) : 28-06-1974 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :-----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقع:

التوقيع:

التوقيع:

تتعمد صحة القرارات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نتیقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: هند محمد احمد محمد National ID: 27406280101221	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



هنا

محمد احمد محمد سليمان

٣ ش عبدالشافى حسن

حدائق القبة - القاهرة

١٩٧٤/٠٦/٢٨

٢٧٤ ٠٦٢٨ ٠١ ٠١٢ ٢١

HV5643059



٢٠١٨/١٢ ٢٧٤٠٦٢٨٠١٠١٢٢١

حاصنة على دبلوم معهد ادارة في سكرتارية

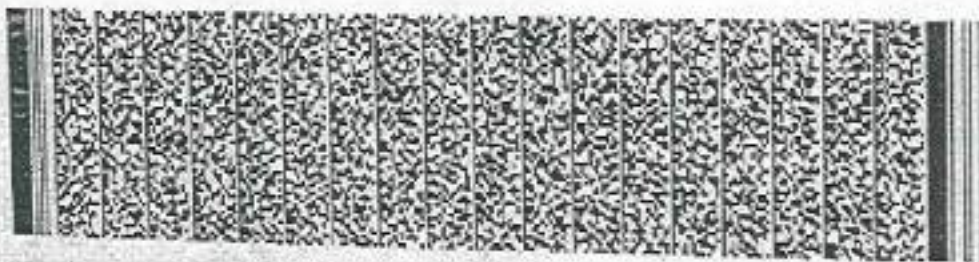
أنتى مسلمة متروجة

احمد محمد السيد حماد

البطاقة سارية حتى ٢٠٢٥/١٢/٢٢



١



Name: هند محمد احمد سليمان			
MR. : 4288			
Detailed Medical Report			
		السيد	
تخية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was diagnosed with right-sided breast cancer and received neoadjuvant chemotherapy consisting of four cycles of doxorubicin and cyclophosphamide (4 ac) followed by twelve weeks of weekly paclitaxel (12 wks taxol). After completion of chemotherapy, the patient underwent breast-conserving surgery with radiation therapy (RT BCS), achieving a clinical complete response (CR) with estrogen-receptor expression 3/8, progesterone-receptor 0/8, and HER2-negative status. Post-operative radiation therapy (PORT) was administered and concluded on 23 April 2026, after a four-week course, and the patient is now scheduled for follow-up at three-month intervals. The patient has completed four cycles of AC and twelve weeks of taxol.			
تحريرا في ,			
التاريخ :		19-09-2026	
الوقت :		mohamed ismael	
		الطبيب المعالج :	
يعتمد ,			

MDT FORM

19/10/2025 G

Name	هند محمد انور محمد	MRN	146-2025
Age	54 yrs	ID	2025-10-19-001
Menopausal status	post menopause	Phone	0111057659
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM	REASON OF MDT	post NACH

Clinical Data

BX: IDC

R T₁ N₂ M₀

ER: 318 PR: 0/8 HER 2: 0 Ki67:



Sonomammography:

TNM

Mets Workup:

Recommendations:

pt Received NACH AC x 4 cycles
Tox/L/Carbo (12wks)
for MDT (surgical)

- for wire localization + WLE & SLNB X according to fast MDT.

MDT OUTCOME & PLAN

WLE → complete clinical response for your appreciated opinion
داعية الى موافقة

WLE + ALND

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
رئيسة	د. محمد حنين	د. محمد حنين			



Patient Name	: الأستاذة / هناد محمد أحمد محمد	Request No.	: 13003997
Patient No.	: 27406280101221	Request Date	: 09/12/2024
Path No.	: 4823/24	Verification Date	: 21/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 50 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data:

Right breast mass.

Nature of specimen:

Tru cut biopsy, collected at 8/12/2024 , 10:27 Am

Gross examination

Multiple greyish white tissue cores up to 2 cm , totally submitted.

Microscopic examination:

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+3+3) Grade III
- * DCIS: Could not be detected within the limits of biopsy recieved
- * Angioinvasion: Could not be detected within the limits of biopsy recieved

Diagnosis:

Right breast mass, tru cut biopsy:
Invasive duct carcinoma, Grade III .
For further IHC (ER,PR,HER2,Ki67).

Prof.Gamal Abdelatty
Dr. Menna Nabil

Verified by :
dr.esraa magdy

Printing Date : 13/01/2025 16:34:37

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذة / خالد محمد احمد محمد	Request No.	: 13003997
Patient No.	: 27406280101221	Request Date	: 09/12/2024
Path No.	: 4823/24	Verification Date	: 28/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 50 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected at 8/12/2024, 10:27 am.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 50% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

IPT:
- ER: Positive (score 3/8).
- PR: Negative (score 0/8).
- Her2: PNegative (score 0).
- Ki67: Positive in 50% of tumor cells.

Verified by :
dr.esraa magdy

Printing Date : 13/01/2025 16:34:41

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Patient Name الأستاذة / هند محمد أحمد محمد
Patient No. 27406280101221
Path No. 4823/24
Sex Female
Age 50 Y
Referring Doc.

Request No. 13003997
Request Date 09/12/2024
Verification Date 28/12/2024
Financial Category أورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is Intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof. Gamal Abdelatty
Dr. Noura Hefny

Verified by :
dr.esraa magdy

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Women's Health Outreach Program

Name: Hend Moh ahmed soliman

Date: 21-11-2024

Bilateral Full field Digital Mammography

Clinical summary: right breast mass .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities (ACR density B).
- Right UOQ speculated irregular shaped dense mass lesion with irregular borders seen in both CC & MLO views
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- AI score gave high 93 percentage of malignancy on right side .
- Ultrasound demonstrated :
- Right UOQ 11 o'clock position , 7 cm FN hypoechoic solid mass lesion with illdefined outline measuring 1.45 x 0.8 x 0.9 cm in maximum dimensions , with positive color Doppler flow
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Right level I multiple pathological axillary lymphnodes with effaced hila in few of them , the largest one seen at level I measuring 3.18 X 1.16 cm another with cystic component some of them showed thickened cortices measuring 7 mm .
- Left breast
 - Scattered fibro glandular densities (ACR density B).
 - No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
 - Ultrasound demonstrated normal parenchyma
 - No solid or cystic lesion
 - Few prominent RA ducts .
 - Normal skin/subcutaneous tissue and nipple/areolar complex.
 - Non-specific axillary LNs.

Impression:

- RIGHT BREAST (BIRADS 5) : RIGHT UOQ SUSPICIOUS MASS LESION WITH MULTIPLE LEVEL I PATHOLOGICAL AXILLARY LYMPHNODES FOR PATHOLOGICAL VERIFICATION & BIOPSY .

- Left breast (BIRADS 2): mild RA prominent ducts .

Recommendations:

Core Biopsy

Much obliged :

Dr / SARAH HUSSEIN .MD

Patient Name: Hend Mohamed Ahmed Mohamed Soliman	Date : 05/10/2025
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID:	Pat. ID: 97268

**Bilateral Digital soft tissue mammography with high resolution
breast sonography revealed:**

Clinical summary: Right breast cancer, post NACT assessment study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ , posterior third tissue marker with no related masses, speculations or microcalcifications yet showing surrounding subtle distortion.
- **Ultrasound** demonstrated No solid or cystic lesions.
- Normal nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): complete radiological response of the previously reported malignant process.

Left breast (BIRADS 1): Normal study.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
AM 11:10 2025/10/05



Patient Name: Hend Mohamed Ahmed Mohamed Soliman	Date : 10/02/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 50 Year(s)
Medical ID: 146/2025	Pat. ID: 97268

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right breast showing UOQ irregular soft tissue mass lesion measuring about 1.7x1.2 cm.
- Right axillary level I , II& III enlarged suspicious lymph nodes the largest seen at level I measuring about 1.4x2.5 cm.
- Left axillary prominent lymphnodes.
- Bilateral prominent supraclavicular lymphnodes.
- clear both lung fields. No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Bilateral elastofibroma dorsi is noted.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of enlarged with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.



Patient Name: Hend Mohamed Ahmed Mohamed Soliman	Date : 10/02/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 50 Year(s)
Medical ID: 146/2025	Pat. ID: 97268

- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- urinary bladder is underfilled.
- Bulky uterus for age with calcified lesion likely calcified fibroid for further US correlation.
- Normal appearance of the other pelvic organs.
- Multiple prominent pelvi abdominal lymphnodes seen paraaortic, aorto caval, mesenteric, bilateral iliac and bilateral inguinal. the largest seen at right iliac measuring about 17x10 mm.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

No definite worrisome lytic or sclerotic lesions

Opinion: Reported case of right breast cancer

- Multiple prominent pelviabdominal lymphnodes for follow up.
- No CT evidence of definite hepatic pulmonary or osseous deposits.

Dr Sarah Magdy
Prof. Menna Talla Hanafy, M.D.

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فند
محمد احمد محمد سليمان
٣٦ سن عبدالشافي حسن
حائلق القبة - القاهرة

٢٧٤٠٦٢٨٠١٠١٢٢١

L06256039



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حصوله على بطوم مهندارة وسكرتارية
متروجة
انثى مسلمة
احمد محمد السيد حماد
البطاقة سارية حتى ٢٠٢٢/٠١/٠١

